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Vertex Pharmaceuticals | North America

Will Inaxaplin work beyond APOL1-mediated FSGS?

Inaxaplin is a key kidney asset in VRTX's pipeline, which is in an ongoing pivotal Ph2/3 trial for APOL1-mediated kidney disease (AMKD), with data 4Q/1Q. But we expect Ph2 AMPLIFIED data in mid-2026, which should offer insight into the potential of the drug beyond FSGS (initial Ph2a POC data).

VRTX's nephrology vertical is taking shape in 2026, driven by two late-stage assets - Pove and Inaxaplin. Recent Pove Ph3 IgAN data represented an important de-risking step for the asset, in our view (see [HERE](#)). On 3/31, VRTX announced the completion of BLA submission to the FDA for potential accelerated approval, and the company expects the review period to be six months from the date of acceptance due to the application of a priority review voucher. For Pove, we model 2035 risk-adjusted sales of \$6.5bn (POS 90%) vs. cons \$3.1bn.

The company is also developing Inaxaplin for APOL1-mediated kidney disease (AMKD). AMKD refers to a spectrum of chronic kidney diseases driven by high-risk variants in the APOL1 gene, which are strongly associated with kidney injury in individuals of African ancestry. Prevalence estimates for AMKD vary widely, and VRTX estimates 150k US+EU patients for primary AMKD and 100k US+EU patients for AMKD with moderate proteinuria or diabetes.

In a small Ph2a POC study, Inaxaplin treatment resulted in ~48% UPCR (biomarker of kidney disease) reduction at week 13 in APOL1-mediated focal segmental glomerulosclerosis (FSGS), which has a strong genetic association, early onset, and rapid progression, but only represents 4% of the AMKD population.

VRTX is running two trials of Inaxaplin in AMKD in different populations, and a key question is whether the drug will work beyond APOL1-mediated FSGS. The ongoing Ph2/3 AMPLITUDE study is in primary AMKD patients (which include FSGS), with data expected in late 2026 or early 2027 and could support an FDA filing. In addition, the ongoing Ph2 AMPLIFIED study (data expected mid-2026) has two cohorts that do not overlap with the population of AMPLITUDE: (1) AMKD with moderate proteinuria (UACR 100-420mg/g); and (2) AMKD with type-2 diabetes (T2D). For inaxaplin, we model 2035 risk-adjusted sales of \$2.6bn (POS 50%) vs. cons \$1.7bn.

How might VRTX trade in response to the Inaxaplin Ph2 AMPLIFIED data? Per VRTX, the AMPLIFIED study has the potential to expand the addressable market to 250k from 150k US+EU patients. We model Inaxaplin sales only into the 150k primary AMKD cohort, and hence believe little value is priced into the stock for AMPLIFIED population. Furthermore, VRTX has recently stated that the biology for AMKD patients with T2D is less clear. That said, we believe investors may look to Cohort 1 (pure AMKD with moderate proteinuria) of the AMPLIFIED study to

MORGAN STANLEY & CO. LLC

Terence C Flynn, Ph.D.
Equity Analyst
Terence.Flynn@morganstanley.com +1 212 761-2230

Chris Yu, J.D., Ph.D.
Equity Analyst
Chris.L.Yu@morganstanley.com +1 212 761-2535

Damien H Kerner
Research Associate
Damien.H.Kerner@morganstanley.com +1 212 761-3829

Alexander Yevdokimov, Ph.D.
Research Associate
Alexander.Yevdokimov@morganstanley.com +1 212 761-2167

Hailey Horowitz
Research Associate
Hailey.Horowitz@morganstanley.com +1 212 761-5264

Connor M Massari
Equity Analyst
Connor.Massari@morganstanley.com +1 212 761-2417

MORGAN STANLEY INDIA COMPANY PRIVATE LIMITED+

Saket Agarwal
Research Associate
Saket.Agarwal@morganstanley.com +91 22 6995-4012

Vertex Pharmaceuticals (VRTX.O, VRTX US)					
Biotechnology United States of America					
Stock Rating	Overweight				
Industry View	Attractive				
Price target	\$612.00				
Shr price, close (Apr 16, 2026)	\$435.65				
Mkt cap, curr (mm)	\$112,409				
52-Week Range	\$510.77-362.50				
Fiscal Year Ending	12/25	12/26e	12/27e	12/28e	
EPS (\$)**	18.53	18.90	20.78	23.82	
Prior EPS (\$)**	-	-	-	-	
P/E	24.5	23.1	21.0	18.3	
EPS (\$)§	18.32	19.05	21.84	25.52	
Div yld (%)	0.0	0.0	0.0	0.0	

Unless otherwise noted, all metrics are based on Morgan Stanley ModelWare framework
** = Based on consensus methodology
§ = Consensus data is provided by Refinitiv Estimates
e = Morgan Stanley Research estimates

QUARTERLY EPS (\$)					
Quarter	2025	2026e Prior	2026e Current	2027e Prior	2027e Current
Q1	4.07	-	4.02	-	-
Q2	4.52	-	4.63	-	-
Q3	4.80	-	5.05	-	-
Q4	5.03	-	5.20	-	-

e = Morgan Stanley Research estimates

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understand whether inaxaplin could work in AMKD patients beyond APOL1-mediated FSGS. Cohort 2 includes patients with AMKD and T2D. Therefore, we provide four potential AMPLIFIED scenarios for the stock to consider. In the event of a positive outcome, we would also pay attention to the breadth of response.

- **Scenario 1.** Inaxaplin reduces UACR by $\geq 30\%$ in both Cohort 1&2 and there is no new safety signal. We would expect VRTX to trade up 5-10%.
- **Scenario 2.** Inaxaplin reduces UACR by $\geq 30\%$ in Cohort 1 only, and there is no new safety signal. We would expect VRTX to trade up low-to-mid-single-digit percentage.
- **Scenario 3.** Inaxaplin fails to reduce UACR by $\geq 30\%$ in both cohorts, but there is no new safety signal. We would expect VRTX to trade down low-to-mid-single-digit percentage.
- **Scenario 4.** There is a new safety signal. We would expect VRTX to trade down high-single-digit percentage.

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Inaxaplin for AMKD

Terence Flynn, PhD

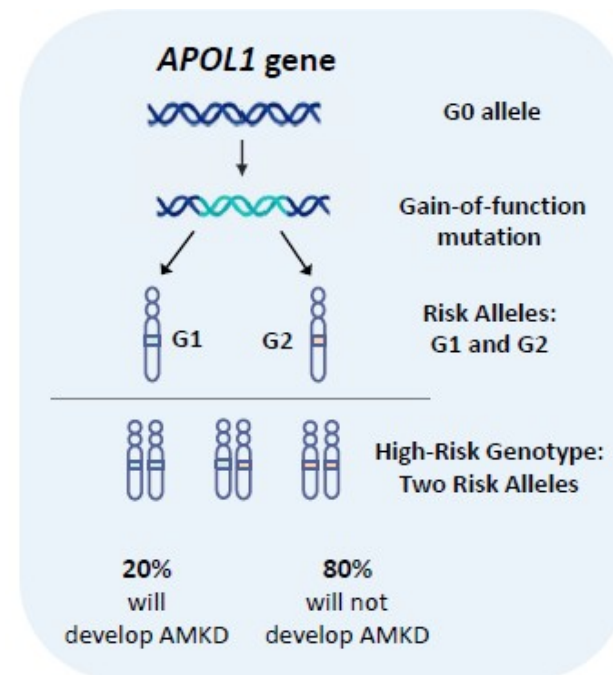
Chris Yu, JD, PhD

4/19/2026

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AMKD Overview

- Primarily affects people of African ancestry. High unmet need due to progressive kidney disease with no approved therapy.
- Per VRTX, African American AMKD patients have a higher rate in decline in eGFR: 6.55 vs 3.63 mL/min/1.73 m²/yr among African Americans with CKD and no APOL1 risk variants.
- G1 and G2 risk alleles define the APOL1 risk genotypes (G1/G1, G2/G2, G1/G2).
- While a high-risk genotype is a necessary factor, a second hit (e.g., viral infections) is further needed for kidney disease to develop.
- There are three main AMKD subpopulations: primary AMKD (including APOL1-mediated FSGS), AMKD with moderate proteinuria (i.e., lower level than primary AMKD), and AMKD with comorbidities (such as T2D and HIV).
- Per Hung (JASN 2024), p.N264K mutation shows that genetic inhibition of APOL1 pore-forming function prevents AMKD, in both diabetics and nondiabetics.

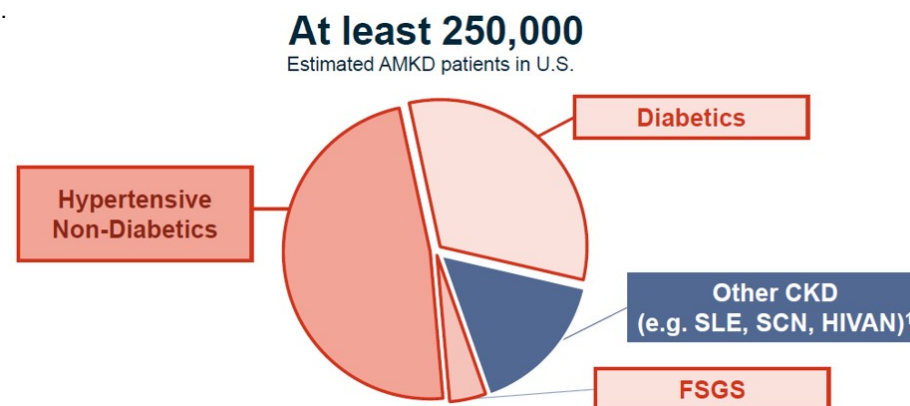


Source: PRIME Education.

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AMKD Market Size

- **We estimate ~1.2mn Americans with AMKD:** Per Srinivasan et al. (Kidney Medicine 2025), in the US, ~13% of African Americans, 6mn people, carry two high-risk alleles. 20% of these patients will develop kidney disease.
- **VRTX estimates 250k US and EU patients:** 150k having primary AMKD patients (addressed by Ph2/3 AMPLITUDE, data expected late 2026 or early 2027) and 100k having AMKD with moderate proteinuria or diabetes (addressed by Ph2 AMPLIFIED, data expected mid-2026).
 - Majority of these patients are not diagnosed. VRTX offers free APOL1 genotyping. CMS also approved ICD-10 codes for AMKD, effective 10/1/2025.
- **MAZE (not covered) estimates at least 250k AMKD patients in the US,** for which diabetic patients make up 40%. See schematic below.
- For inaxaplin, we model 2035 risk-adjusted sales of \$2.6bn (POS 50%) vs. cons \$1.7bn. Our 2035 sales assume ~38k patients on therapy (25% of the 150k patients).
- Potential pricing analog: TVTX (not covered)'s Filspari has an annual WAC of ~\$150k, and the GTN is in the mid-20% range. We model \$135k annual net price.



Source: Company data (1. SLE = systemic lupus erythematosus HIVAN = HIV-associated nephropathy, SCN = sickle cell nephropathy, FSGS = Focal Segmental Glomerulosclerosis)

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VRTX’s Inaxaplin Ph2a Data in APOL1-mediated FSGS

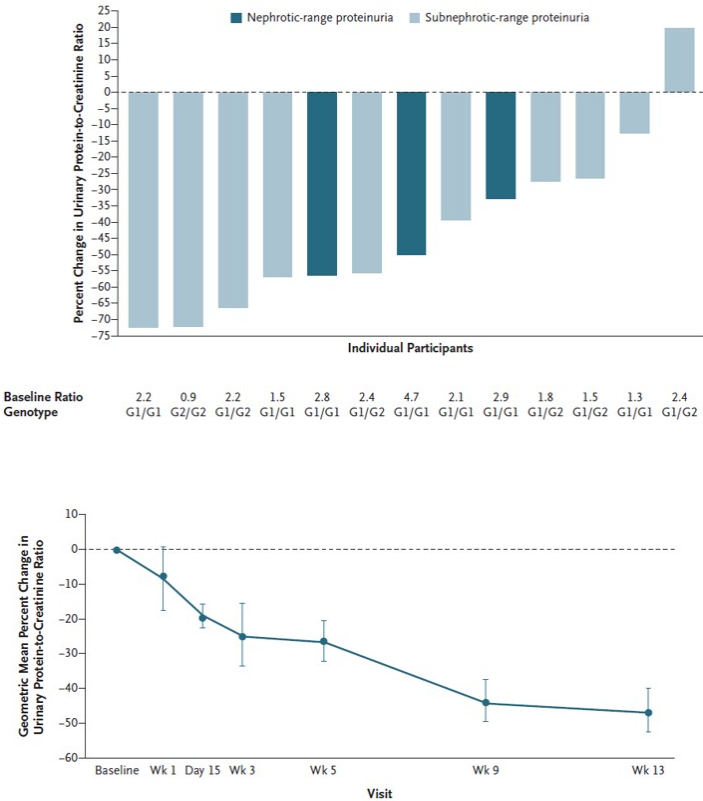
- Inaxaplin is an oral, small-molecule inhibitor of APOL1. In the Ph2a study, it was administered 15mg QD for 2 weeks, followed by 45mg QD for 11 weeks.
- In APOL1-mediated FSGS patients (n=13), mean UPCR reduction of ~48% at week 13, which is equivalent to mean UACR reduction of 43% at week 13. For the overall population, 50%/44% were on ACEi/ARB.
- Urine Protein-Creatinine Ratio (UPCR) and Urine Albumin-Creatinine Ratio (UACR) are biomarkers of kidney damage. UPCR measures total protein in the urine, while UACR measures albumin specifically, so the latter is more sensitive for detecting early kidney damage.
- Nephrotic-range proteinuria was defined as a UPCR of at least 2.7 and less than 10, and subnephrotic-range proteinuria as a UPCR of at least 0.7 and less than 2.7.

Variable	Total (N=13)	Patients with nephrotic-range proteinuria (N=3)	Patients with subnephrotic-range proteinuria (N=10)
Mean UPCR			
At baseline	2.21±0.95	3.47±1.07	1.84±0.52
At wk13	1.27±0.73	1.83±0.58	1.10±0.71
Geometric % change from baseline at wk13	-47.6	-47.7	-47.5

Source: Egbuna (NEJM 2023)

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Additional Inaxaplin Ph2a FSGS Data



Source: Egbuna (NEJM 2023)

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Inaxaplin Ph2 Safety Data

- Inaxaplin has an acceptable safety profile (see table), but longer-term data from a larger population is needed to fully derisk the profile.
- An Independent Data Monitoring Committee (IDMC) previously reviewed unblinded Ph2 safety and efficacy data (N=66) from the Phase 2/3 AMPLITUDE trial and recommended enrolling adolescents with AMKD ages 10-17 years in the Ph3 portion of the study, which suggests to us that the safety profile up until then has been acceptable.
- The IDMC also recommended the selection of 45mg QD inaxaplin dose for the expanded Ph3 portion.
- Theoretically, individuals lacking APOL1 can still maintain normal kidney function suggests that this gene is not essential for normal kidney development or functioning.

Event	Total (N=16)	Participants with Nephrotic-Range Proteinuria (N=3)	Participants with Subnephrotic-Range Proteinuria (N=13)
Any adverse event†	15 (94)	3 (100)	12 (92)
Adverse events according to severity			
Mild	7 (44)	1 (33)	6 (46)
Moderate	8 (50)	2 (67)	6 (46)
Severe	0	0	0
Life-threatening	0	0	0
Serious adverse event‡	1 (6)	0	1 (8)
Adverse event leading to treatment discontinuation	0	0	0
Adverse event occurring in ≥2 participants			
Headache	4 (25)	1 (33)	3 (23)
Back pain	3 (19)	0	3 (23)
Nausea	3 (19)	1 (33)	2 (15)
Decrease in blood bicarbonate level	2 (12)	0	2 (15)
Diarrhea	2 (12)	1 (33)	1 (8)
Dizziness	2 (12)	0	2 (15)
Dyspepsia	2 (12)	0	2 (15)
Fatigue	2 (12)	0	2 (15)

† Adverse events that were considered by the investigator as being possibly related to inaxaplin occurred in three participants: headache in one participant in the nephrotic-range group and in one in the subnephrotic-range group and headache, abdominal distension, dyspepsia, and back pain in one participant in the subnephrotic-range group.

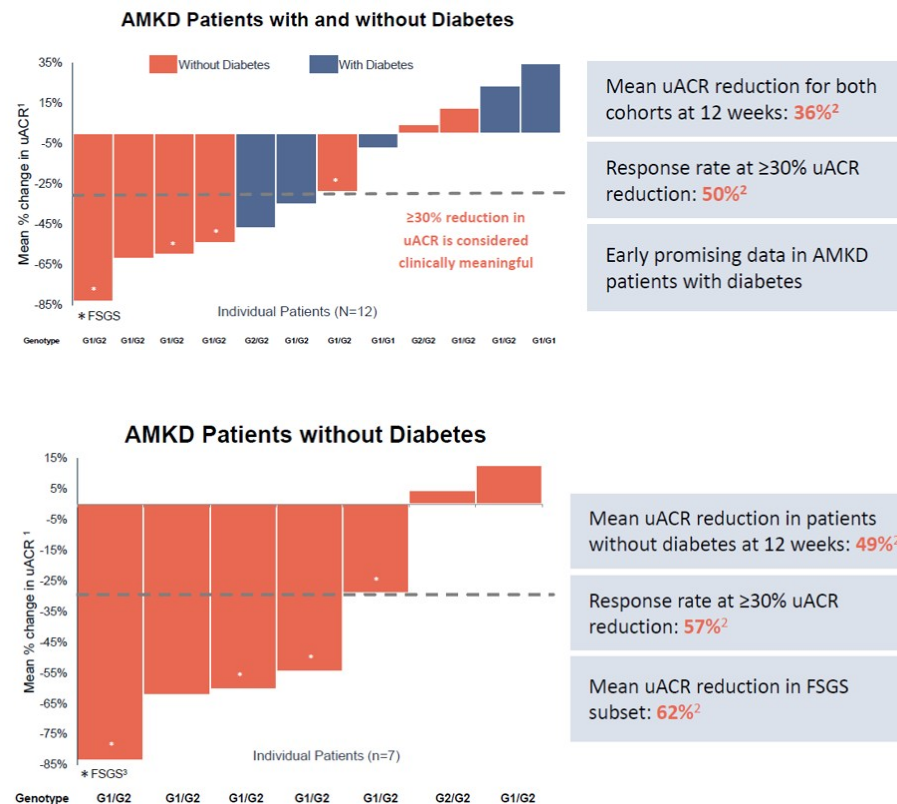
‡ Two serious adverse events occurred in one participant: deep-vein thrombosis and uterine leiomyoma. Neither event was considered by the investigator to be related to inaxaplin.

Source: Egbuna (NEJM 2023)

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MAZE's Ph2 Data in AMKD with or w/o Diabetes

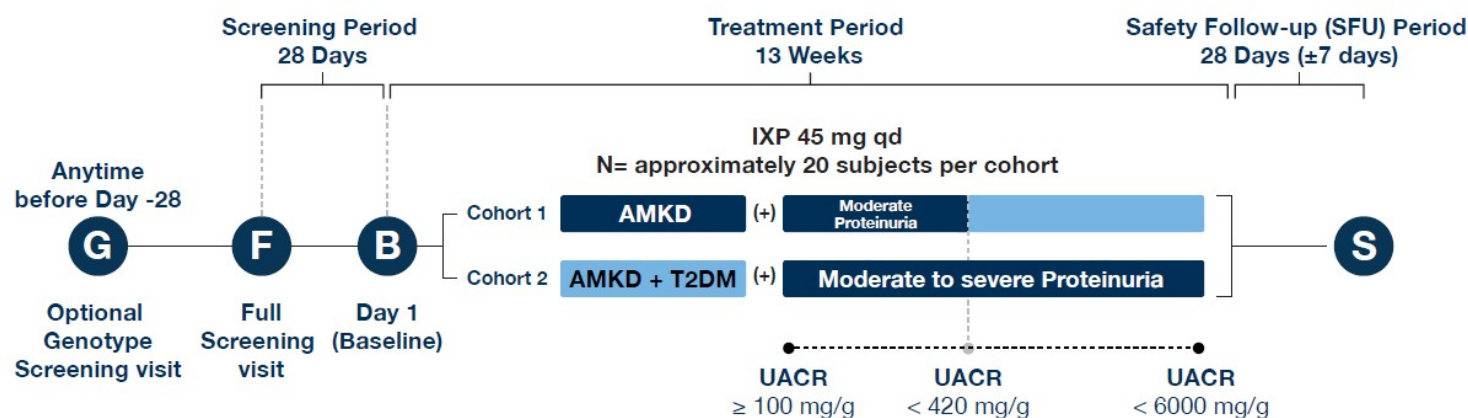
- MZE829 (APOL1 inhibitor) is dosed QD 250mg oral. Potential to use a higher dose in a pivotal study.
- Key inclusion criteria: patients on stable background CKD regimen for at least 8 weeks prior to study.
- 1/3 of the evaluable patients have FSGS.
- 42% of the evaluable patients have diabetes.
- Baseline UACR characteristics
 - Total AMKD: 1.1 g/g
 - AMKD w/o diabetes: 1.4 g/g
 - AMKD w/ diabetes: 0.8 g/g
- Baseline UPCR characteristics
 - Total AMKD: 1.7 g/g
 - AMKD w/o diabetes: 2.1 g/g
 - AMKD w/ diabetes: 1.4 g/g



Source: MAZE presentation

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Inaxaplin's Ph2 AMPLIFIED Study



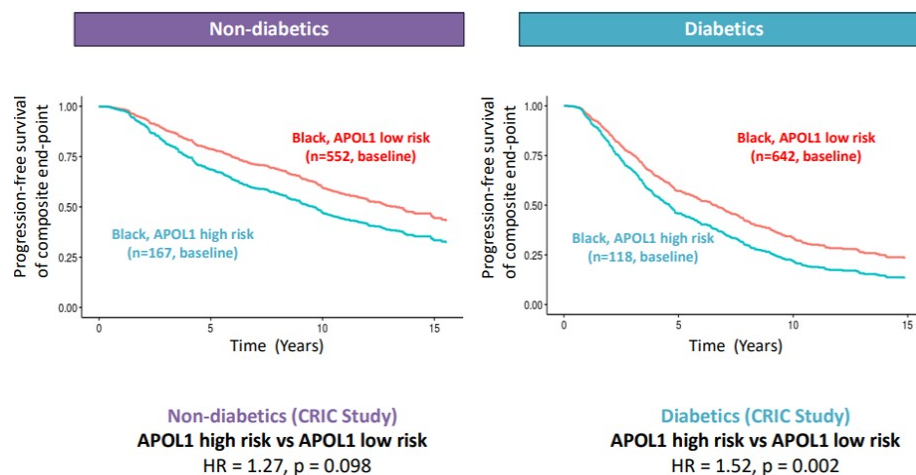
- Data expected mid-2026 (the trial is recruiting patients as of 4/14/26).
- Primary endpoint: % change in UACR at week 13. Secondary endpoint: safety and tolerability.
- The trial has two cohorts: (1) AMKD with moderate proteinuria (UACR 100-420mg/g); and (2) AMKD with T2D. There is no overlap between these patient populations and that in Inaxaplin's Ph3 AMPLITUDE study (i.e., pure AMKD with more severe proteinuria – UPCR 0.7-10 g/g).
- Per VRTX, the role of APOL1 is less clear in AMKD patients with comorbidities (e.g., T2D). For example, it is not clear if the treatment of diabetes with background medications would lower UACR and thus treat the kidney disease.
- Decision on a second pivotal study in a separate population will depend on the outcomes of both AMPLIFIED and AMPLITUDE.

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What Do We Want to Learn from Ph2 AMPLIFIED Data?

The AMPLIFIED study is about expanding the potential market opportunity for inaxaplin. There is a lack of overlap between the patient populations, so we expect a limited read-through from AMPLIFIED to AMPLITUDE. However, we believe some investors may look to Cohort 1 (pure AMKD with moderate proteinuria) to gauge POS of AMPLITUDE, which includes patients beyond FSGS. We are focused on the following for AMPLIFIED:

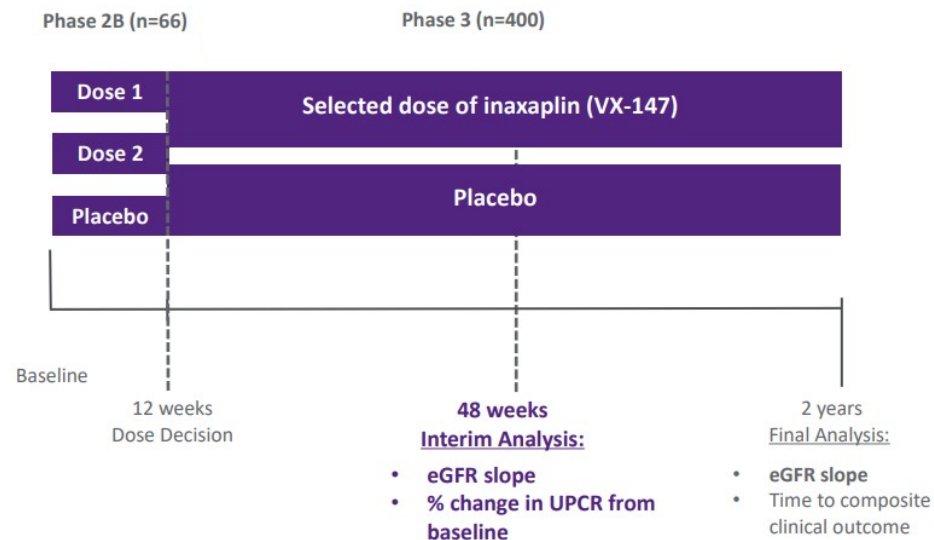
- Can inaxaplin reduce UACR by $\geq 30\%$ in AMKD patients with moderate proteinuria? Egbuna (NEJM 2023) noted that a 30% reduction in UACR translated to a 27% reduction in the risk of a composite clinical endpoint of ESKD.
- Can inaxaplin reduce UACR by $\geq 30\%$ in AMKD patients with T2D? Any efficacy difference due to background therapy? APOL1 high risk genotype was found to increase risk of progression to ESKD in both diabetics and non-diabetics. See below.
- Does the additional safety data confirm inaxaplin's profile to date?



Source: Company data

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Inaxaplin Ph2/3 AMPLITUDE Study



- **Data expected late 2026 or early 2027 (selected dose is 45mg QD oral).**
- Key inclusion criteria: UPCR = 0.7 to < 10 g/g, and eGFR = 25 to < 75 mL/min/1.73 m².
- Per VRTX, eGFR (not UPCR) data is FDA's focus of the Ph2/3 interim analysis.
- **What is a positive Ph3 outcome?** We believe a stat sig improvement on eGFR over placebo a positive outcome given the lack of approved therapies for AMKD and prior physician commentary. Egbuna (NEJM 2023) notes that a reduction in proteinuria of more than 40% is clinically meaningful based on data from nearly 30k patients. As a result, we think at least 40% UPCR reduction from baseline is likely positive.

Source: VRTX Presentation; Chertow Abstract.

Valuation Methodology and Risks

Vertex Pharmaceuticals (VRTX.O)

Our Price Target Is Derived From A Discounted Cash Flow Analysis Using A WACC Of 10% And A Terminal Growth Rate Of 2% Post 2040.

Risks to Upside

- Casgevy takes a significant market share
- Journavx takes a significant share
- Inaxaplin has a positive Ph3 interim readout
- Journavx has a positive Ph3 readout in DPN

Risks to Downside

- Casgevy/Journavx fails to take meaningful market share
- Journavx Ph3 trial in DPN fails
- Inaxaplin fails in Ph3
- Pove fails to take meaningful share in IgAN

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Global Stock Ratings Distribution

(as of March 31, 2026)

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Stock Rating Category	Coverage Universe		Investment Banking Clients (IBC)			Other Material Investment Services Clients (MISC)	
	Count	% of Total	Count	% of Total IBC	% of Rating Category	Count	% of Total Other MISC
Overweight/Buy	1534	42%	461	50%	30%	698	43%
Equal-weight/Hold	1573	43%	372	40%	24%	716	44%
Not-Rated/Hold	4	0%	1	0%	25%	1	0%
Underweight/Sell	568	15%	89	10%	16%	209	13%
Total	3,679		923			1624	

Data include common stock and ADRs currently assigned ratings. Investment Banking Clients are companies from whom Morgan Stanley received investment banking compensation in the last 12 months. Due to rounding off of decimals, the percentages provided in the "% of total" column may not add up to exactly 100 percent.

Analyst Stock Ratings

Overweight (O). The stock's total return is expected to exceed the average total return of the analyst's industry (or industry team's) coverage universe, on a risk-adjusted basis, over the next 12-18 months.

Equal-weight (E). The stock's total return is expected to be in line with the average total return of the analyst's industry (or industry team's) coverage universe, on a risk-adjusted basis, over the next 12-18 months.

Not-Rated (NR). Currently the analyst does not have adequate conviction about the stock's total return relative to the average total return of the analyst's industry (or industry team's) coverage universe, on a risk-adjusted basis, over the next 12-18 months.

Underweight (U). The stock's total return is expected to be below the average total return of the analyst's industry (or industry team's) coverage universe, on a risk-adjusted basis, over the next 12-18 months.

Unless otherwise specified, the time frame for price targets included in Morgan Stanley Research is 12 to 18 months.

Analyst Industry Views

Attractive (A): The analyst expects the performance of his or her industry coverage universe over the next 12-18 months to be attractive vs. the relevant broad market benchmark, as indicated below.

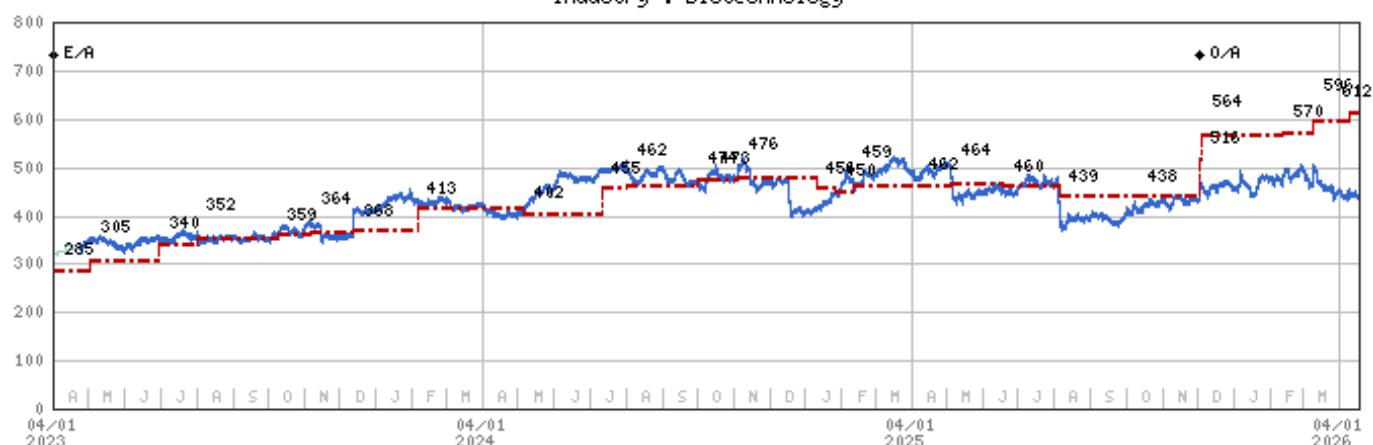
In-Line (I): The analyst expects the performance of his or her industry coverage universe over the next 12-18 months to be in line with the relevant broad market benchmark, as indicated below.

Cautious (C): The analyst views the performance of his or her industry coverage universe over the next 12-18 months with caution vs. the relevant broad market benchmark, as indicated below.

Benchmarks for each region are as follows: North America - S&P 500; Latin America - relevant MSCI country index or MSCI Latin America Index; Europe - MSCI Europe; Japan - TOPIX; Asia - relevant MSCI country index or MSCI sub-regional index or MSCI AC Asia Pacific ex Japan Index.

Stock Price, Price Target and Rating History (See Rating Definitions)

Vertex Pharmaceuticals (VRTX.0) - As of 04/17/26 GMT in USD
Industry : Biotechnology



Stock Rating History: 4/1/21 : E/I; 9/7/21 : U/I; 5/3/22 : E/I; 11/5/22 : E/A; 12/3/25 : O/A

Price Target History: 1/19/21 : 251; 4/16/21 : 253; 4/30/21 : 254; 6/11/21 : 205; 7/30/21 : 202; 10/12/21 : 201; 1/18/22 : 203; 1/27/22 : 208; 4/12/22 : 222; 5/3/22 : 250; 8/4/22 : 253; 10/27/22 : 285; 1/23/23 : 286; 2/8/23 : 285; 5/2/23 : 305; 6/30/23 : 340; 8/1/23 : 352; 10/9/23 : 359; 11/7/23 : 364; 12/12/23 : 368; 2/5/24 : 413; 5/6/24 : 402; 7/11/24 : 455; 8/1/24 : 462; 10/1/24 : 474; 10/11/24 : 473; 11/4/24 : 476; 1/10/25 : 456; 1/26/25 : 450; 2/10/25 : 459; 4/8/25 : 462; 5/5/25 : 464; 6/18/25 : 460; 8/5/25 : 439; 10/10/25 : 438; 12/3/25 : 516; 12/4/25 : 564; 2/12/26 : 570; 3/9/26 : 596; 4/10/26 : 612

Source: Morgan Stanley Research Date Format : MM/DD/YY Price Target --- No Price Target Assigned (NA)

Stock Price (Not Covered by Current Analyst) — Stock Price (Covered by Current Analyst) —

Stock and Industry Ratings (abbreviations below) appear as ♦ Stock Rating/Industry View

Stock Ratings: Overweight (O) Equal-weight (E) Underweight (U) Not-Rated (NR) No Rating Available (NA)

Industry View: Attractive (A) In-line (I) Cautious (C) No Rating (NR)

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Effective January 13, 2014, the industry view benchmarks for Morgan Stanley Asia Pacific are as follows: relevant MSCI country index or MSCI sub-regional index or MSCI AC Asia Pacific ex Japan Index.

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COMPANY (TICKER)	RATING (AS OF)	PRICE* (04/16/2026)
Judah C. Frommer, CFA		
4D Molecular Therapeutics Inc (FDMT.O)	E (11/06/2025)	\$10.06
Abivax (ABVX.O)	O (07/23/2025)	\$122.47
AgomAb Therapeutics NV (AGMB.O)	O (03/03/2026)	\$11.20
Apellis Pharmaceuticals Inc (APLS.O)	E (11/21/2024)	\$40.87
Arcellx Inc (ACLX.O)	++	\$115.10
Arcutis Biotherapeutics, Inc. (ARQT.O)	O (07/02/2025)	\$24.19
Belite Bio Inc (BLTE.O)	O (01/06/2026)	\$158.05
Bicara Therapeutics (BCAX.O)	O (10/08/2024)	\$23.93
Celldex Therapeutics Inc (CLDX.O)	O (03/20/2025)	\$33.86
Compass Pathways PLC (CMPS.O)	O (08/08/2025)	\$6.66
enGene Holdings Inc. (ENGN.O)	O (03/08/2024)	\$8.08
Evomune Inc (EVMN.N)	O (12/01/2025)	\$26.77
Galapagos NV (GLPG.O)		\$28.86
Genmab A/S (GMAB.O)	E (02/16/2026)	\$28.30
Immix Biopharma (IMMX.O)	O (03/25/2026)	\$9.96
Incyte Corp (INCY.O)	E (07/02/2025)	\$95.32
Kalaris Therapeutics (KLRS.O)	O (04/16/2026)	\$6.59
Kymera Therapeutics Inc (KYMR.O)	O (07/02/2025)	\$86.14
ProKidney Corp (PROK.O)	E (11/10/2022)	\$1.95

PTC Therapeutics (PTCT.O)	O (03/06/2025)	\$71.70
Regenxbio Inc (RGNX.O)	O (11/15/2024)	\$9.19
Trevi Therapeutics Inc (TRVI.O)	O (08/21/2025)	\$14.54
Zenas Biopharma Inc (ZBIO.O)	E (01/05/2026)	\$21.76
Maxwell Skor		
Adagene Inc. (ADAG.O)	E (01/30/2025)	\$3.85
Amicus Therapeutics Inc. (FOLD.O)	++	\$14.45
Ascendis Pharma A/S (ASND.O)	O (05/05/2025)	\$242.95
Atea Pharmaceuticals Inc (AVIR.O)	E (08/12/2024)	\$5.70
Bicycle Therapeutics Plc (BCYC.O)	E (02/14/2022)	\$5.14
Crinetix Pharmaceuticals (CRNX.O)	O (01/16/2024)	\$39.02
Cytokinetix Inc (CYTK.O)	O (02/13/2025)	\$65.42
Insmid Inc (INSM.O)	O (03/30/2026)	\$142.79
Monopar Therapeutics (MNPR.O)	O (01/09/2026)	\$57.70
Pharvaris N.V. (PHVS.O)	O (08/15/2023)	\$28.78
Sana Biotechnology, Inc (SANA.O)	O (03/01/2021)	\$3.47
Tscan Therapeutics Inc (TCRX.O)	E (11/13/2025)	\$1.08
Ultragenyx Pharmaceutical Inc (RARE.O)	O (03/27/2019)	\$24.52
Michael E Ulz		
Aardvark Therapeutics Inc. (AARD.O)	E (03/02/2026)	\$4.99
Alnylam Pharmaceuticals Inc (ALNY.O)	E (09/09/2022)	\$320.13
Arrowhead Pharmaceuticals Inc (ARWR.O)	E (09/09/2022)	\$67.98
BioAge Labs Inc (BIOA.O)	E (12/05/2025)	\$16.99
Cabaletta Bio Inc (CABA.O)	O (01/27/2023)	\$3.52
Dyne Therapeutics Inc (DYN.O)	O (04/30/2024)	\$19.91
Fractyl Health Inc (GUTS.O)	E (01/29/2026)	\$0.51
Ionis Pharmaceuticals Inc (IONS.O)	O (07/30/2025)	\$75.26
Kyverna Therapeutics (KYTX.O)	O (03/04/2024)	\$9.53
Mirum Pharmaceuticals (MIRM.O)	O (11/13/2023)	\$95.86
Rhythm Pharmaceuticals Inc (RYTM.O)	O (12/19/2023)	\$88.11
Rocket Pharmaceuticals Inc (RCKT.O)	E (05/27/2025)	\$3.90
Sarepta Therapeutics Inc (SRPT.O)	E (06/16/2025)	\$21.17
Silence Therapeutics Plc (SLN.O)	O (05/08/2023)	\$7.60
Tenaya Therapeutics Inc (TNYA.O)	O (08/24/2021)	\$0.75
Viking Therapeutics Inc (VKTX.O)	O (06/27/2024)	\$35.51
Vir Biotechnology Inc (VIR.O)	O (01/08/2025)	\$10.33
Zentalis Pharmaceuticals Inc (ZNTL.O)	E (06/18/2024)	\$5.02
Sean Laaman, Ph.D.		
ABSCI CORP (ABSI.O)	E (01/08/2026)	\$3.23
Acadia Pharmaceuticals Inc (ACAD.O)	E (08/06/2024)	\$21.25
Alector Inc (ALEC.O)	U (11/26/2024)	\$2.55
argenx SE (ARGX.O)	O (07/02/2025)	\$828.35
Axsome Therapeutics (AXSM.O)	E (01/08/2026)	\$184.19
BeOne Medicines (ONC.O)	O (12/02/2024)	\$320.70
Biomarin Pharmaceutical Inc (BMRN.O)	++	\$54.10
BridgeBio Oncology Therapeutics (BBOT.O)	O (12/05/2025)	\$9.22
BridgeBio Pharma Inc (BBIO.O)	O (01/06/2026)	\$77.51
Centessa Pharmaceuticals, Inc (CNTA.O)	++	\$39.53
CG Oncology (CGON.O)	O (02/19/2024)	\$66.34
Contineum Therapeutics (CTNM.O)	E (01/08/2026)	\$13.30
Cullinan Therapeutics (CGEM.O)	O (03/06/2025)	\$14.74
Denali Therapeutics Inc (DNLI.O)	O (03/06/2025)	\$20.71
Disc Medicine Inc (IRON.O)	O (11/04/2024)	\$69.45
Eikon Therapeutics Inc (EIKN.O)	O (03/02/2026)	\$11.24
Erasca, Inc. (ERAS.O)	E (08/17/2025)	\$18.23

Exelixis Inc. (EXEL.O)	E (01/08/2026)	\$44.38
Generate Biomedicines Inc (GENB.O)	O (03/24/2026)	\$12.61
Halozyme Therapeutics, Inc (HALO.O)	O (08/05/2025)	\$66.64
Immunocore Holdings Ltd (IMCR.O)	E (12/12/2024)	\$31.38
MapLight Therapeutics (MPLT.O)	O (11/21/2025)	\$28.51
Neurocrine Biosciences Inc (NBIX.O)	E (01/08/2026)	\$128.41
Recursion Pharmaceuticals Inc (RXRX.O)	E (05/22/2023)	\$3.60
Schrodinger Inc. (SDGR.O)	E (11/19/2021)	\$12.06
Terence C Flynn, Ph.D.		
Alumis Inc. (ALMS.O)	O (05/30/2025)	\$25.00
Amgen Inc. (AMGN.O)	E (10/16/2023)	\$349.39
Arcus Biosciences Inc. (RCUS.N)	E (01/08/2026)	\$23.59
Arvinas Inc (ARVN.O)	E (04/06/2022)	\$11.00
Biogen Inc (BIIB.O)	E (10/31/2024)	\$176.02
Biohaven Ltd (BHAVN.N)	O (07/24/2024)	\$10.01
BioNTech SE (BNTX.O)	O (09/23/2024)	\$102.12
CRISPR Therapeutics AG (CRSP.O)	U (10/11/2022)	\$55.72
Gilead Sciences Inc. (GILD.O)	O (01/10/2025)	\$138.55
Intellia Therapeutics Inc (NTLA.O)	E (01/26/2025)	\$14.16
Legend Biotech Corp (LEGN.O)	O (01/31/2022)	\$21.08
Moderna Inc (MRNA.O)	E (12/16/2020)	\$54.68
Nurix Therapeutics Inc. (NRIX.O)	O (01/08/2026)	\$16.69
Prime Medicine Inc (PRME.O)	E (11/14/2022)	\$3.67
Regeneron Pharmaceuticals Inc. (REGN.O)	E (12/03/2025)	\$746.00
Structure Therapeutics Inc (GPCR.O)	O (09/22/2024)	\$53.50
United Therapeutics Corp (UTHR.O)	E (07/11/2024)	\$580.51
Vertex Pharmaceuticals (VRTX.O)	O (12/03/2025)	\$435.65

Stock Ratings are subject to change. Please see latest research for each company.

* Historical prices are not split adjusted.